

Paws & Claws Veterinary Practice

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Attending Veterinarian: **Dr. Lena Park, DVM**

FOLLOW-UP NOTE — CHRONIC DISEASE MANAGEMENT

Note Type:	FOLLOW_UP	Date of Visit:	2024-08-14
Patient:	Mochi	Owner:	Sam Rivera
Species:	Feline	Breed:	Ragdoll
DOB:	2016-07-19	Weight:	6.2 kg

REASON FOR VISIT

Mochi presents for 6-week diabetic recheck. Owner reports good compliance with insulin protocol. Patient diagnosed with diabetes mellitus type 2 in January 2024 (pre-existing). Currently on Vetsulin (porcine insulin zinc suspension) 2 IU SC BID. Owner also managing concurrent chronic kidney disease Stage 2 (CKD-II, diagnosed March 2024).

INTERVAL HISTORY

Owner reports Mochi's appetite has been good. Water intake has decreased compared to pre-insulin levels — consistent with improving glycaemic control. No hypoglycaemic episodes observed (no collapse, trembling, or disorientation). Weight is stable — owner has been feeding renal diet (Hill's k/d) as recommended. Urination frequency reduced since last visit. Activity level improved; more playful. No vomiting or diarrhoea.

PHYSICAL EXAMINATION

Parameter	Value	Reference Range	Status
Temperature	38.3 C	38.0–39.5 C	Normal
Heart rate	168 bpm	140–200 bpm	Normal
Weight	6.2 kg	—	Stable (+0.1 kg from last visit)
BCS	4/9	—	Thin — monitor
Hydration	Normal	< 5% dehydrated	Normal

General: BAR. Coat slightly unkempt but improved from last visit. Muscle mass: mild temporal and epaxial wasting. Oral: Grade 2 tartar; mild gingivitis. Eyes: Mild bilateral lenticular sclerosis. Kidneys: palpably small bilaterally, consistent with CKD. No pain on renal palpation. Bladder: not distended.

DIAGNOSTIC RESULTS

Fructosamine (in-house): 320 umol/L (reference for well-controlled diabetic cat: 350–450 umol/L — slightly below target range; may indicate episodes of mild hypoglycaemia or improving insulin sensitivity). Urine glucose (dipstick): 2+ (expected while establishing control). BUN: 38 mg/dL (elevated, ref 16–36) | Creatinine: 2.1 mg/dL (elevated, ref 0.8–1.8) | Phosphorus: 4.9 mg/dL (high-normal, ref 3.1–7.5) — stable from prior panel. SDMA: 18 ug/dL (consistent with IRIS Stage 2 CKD). UPC (urine protein:creatinine ratio, submitted to external lab 2 weeks ago): 0.32 (borderline proteinuria).

ASSESSMENT

1. Diabetes mellitus, type 2 (pre-existing) — feline; improving glycaemic control based on fructosamine trending downward; slight undershoot suggests possible insulin dose reduction may be appropriate. 2. Chronic kidney disease, IRIS Stage 2 (pre-existing) — stable creatinine and SDMA; borderline proteinuria warrants monitoring. 3. Mild dental disease (Grade 2 gingivitis / tartar) — owner advised to schedule dental cleaning when metabolic status permits. 4. Mild muscle wasting —

secondary to diabetes and CKD; nutritional support discussed.

PLAN

- Reduce Vetsulin dose to 1.5 IU SC BID given fructosamine trending below target. Re-check fructosamine in 6 weeks. - Continue Hill's k/d renal diet. Ensure adequate caloric intake given BCS 4/9. - Continue phosphorus binder (Epakitin) in food SID. - Renal panel recheck in 3 months; UPC recheck in 6 weeks. - Dental cleaning to be scheduled when DM well-controlled and renal status permits (target fructosamine 350–450 prior to scheduling). - Owner educated on hypoglycaemia recognition and emergency corn syrup protocol. - Next DM recheck: 6 weeks. Urine culture to be considered if proteinuria worsens.

EXPECTED CODING OUTPUT (test validation reference)

Expected Code	System	Category	Confidence Tier
E11.9	ICD-10-CM	DIAGNOSIS	HIGH
N18.2	ICD-10-CM	DIAGNOSIS	HIGH
K05.10	ICD-10-CM	DIAGNOSIS	HIGH
M62.50	ICD-10-CM	DIAGNOSIS	MEDIUM
R80.9	ICD-10-CM	SYMPTOM	MEDIUM

Test assertion: DM coded as E11.9 (feline = type 2 analogue, CR-04 applied) | both DM and CKD flagged is_pre_existing = true | requires_review = false | overall_confidence = MEDIUM (muscle wasting inference) | hypoglycaemia NOT coded (potential episode mentioned but negated by owner report)